

HOUSE BILL No. 1255

DIGEST OF INTRODUCED BILL

Citations Affected: IC 16-36-1-5.

Synopsis: Presumption of continuation of life. Establishes a presumption that the continuation of life is in a minor's best interests. Requires a health care provider to obtain the consent of each parent or each legal guardian before issuing a do not resuscitate order or otherwise withholding or withdrawing treatment to allow the natural death of a minor. Prohibits a health care provider from interfering with the transfer of a minor patient at the request of a parent or guardian or otherwise preventing life saving measures before or during the transfer. States that a court does not have jurisdiction to withdraw life sustaining treatment for a minor.

Effective: July 1, 2026.

Sweet, Patterson, Cash

January 5, 2026, read first time and referred to Committee on Public Health.



Second Regular Session of the 124th General Assembly (2026)

PRINTING CODE. Amendments: Whenever an existing statute (or a section of the Indiana Constitution) is being amended, the text of the existing provision will appear in this style type, additions will appear in **this style type**, and deletions will appear in ~~this style type~~.

Additions: Whenever a new statutory provision is being enacted (or a new constitutional provision adopted), the text of the new provision will appear in **this style type**. Also, the word **NEW** will appear in that style type in the introductory clause of each SECTION that adds a new provision to the Indiana Code or the Indiana Constitution.

Conflict reconciliation: Text in a statute in *this style type* or ~~this style type~~ reconciles conflicts between statutes enacted by the 2025 Regular Session of the General Assembly.

HOUSE BILL No. 1255

A BILL FOR AN ACT to amend the Indiana Code concerning health.

Be it enacted by the General Assembly of the State of Indiana:

- 1 SECTION 1. IC 16-36-1-5, AS AMENDED BY P.L.67-2018,
- 2 SECTION 3, IS AMENDED TO READ AS FOLLOWS [EFFECTIVE
- 3 JULY 1, 2026]: Sec. 5. (a) If an adult incapable of consenting under
- 4 section 4 of this chapter has not appointed a health care representative
- 5 under section 7 of this chapter or the health care representative
- 6 appointed under section 7 of this chapter is not reasonably available or
- 7 declines to act, except as provided in sections 9 and 9.5 of this chapter,
- 8 consent to health care may be given in the following order of priority:
- 9 (1) A judicially appointed guardian of the person or a
- 10 representative appointed under section 8 of this chapter.
- 11 (2) A spouse.
- 12 (3) An adult child.
- 13 (4) A parent.
- 14 (5) An adult sibling.
- 15 (6) A grandparent.
- 16 (7) An adult grandchild.
- 17 (8) The nearest other adult relative in the next degree of kinship



who is not listed in subdivisions (2) through (7).

(9) A friend who:

(A) is an adult;

(B) has maintained regular contact with the individual; and

(C) is familiar with the individual's activities, health, and religious or moral beliefs.

(10) The individual's religious superior, if the individual is a member of a religious order.

(b) **Except as provided in subsection (f)**, consent to health care for a minor not authorized to consent under section 3 of this chapter may be given by any of the following:

(1) A judicially appointed guardian of the person or a representative appointed under section 8 of this chapter.

(2) A parent or an individual in loco parentis if:

(A) there is no guardian or other representative described in subdivision (1);

(B) the guardian or other representative is not reasonably available or declines to act; or

(C) the existence of the guardian or other representative is unknown to the health care provider.

(3) An adult sibling of the minor if:

(A) there is no guardian or other representative described in subdivision (1);

(B) a parent or an individual in loco parentis is not reasonably available or declines to act; or

(C) the existence of the parent or individual in loco parentis is unknown to the health care provider after reasonable efforts are made by the health care provider to determine whether the minor has a parent or an individual in loco parentis who is able to consent to the treatment of the minor.

(4) A grandparent of the minor if:

(A) there is no guardian or other representative described in subdivision (1);

(B) a parent, an individual in loco parentis, or an adult sibling is not reasonably available or declines to act; or

(C) the existence of the parent, individual in loco parentis, or adult sibling is unknown to the health care provider after reasonable efforts are made by the health care provider to determine whether the minor has a parent, an individual in loco parentis, or an adult sibling who is able to consent to the treatment of the minor.

(c) A representative delegated authority to consent under section 6



1 of this chapter has the same authority and responsibility as the
2 individual delegating the authority.

3 (d) An individual authorized to consent for another under this
4 section shall act in good faith and in the best interest of the individual
5 incapable of consenting.

6 (e) If there are multiple individuals at the same priority level under
7 this section, those individuals shall make a reasonable effort to reach
8 a consensus as to the health care decisions on behalf of the individual
9 who is unable to provide health care consent. If the individuals at the
10 same priority level disagree as to the health care decisions on behalf of
11 the individual who is unable to provide health care consent, a majority
12 of the available individuals at the same priority level controls.

13 **(f) This subsection does not apply to a minor described in**
14 **section 3(a)(2) of this chapter. It is hereby established that there is**
15 **a presumption that the continuation of life is in a minor's best**
16 **interests, including a minor who is born premature at less than**
17 **twenty (20) weeks gestation. A health care provider may not issue**
18 **a do not resuscitate order or otherwise withhold or withdraw**
19 **treatment that would allow the natural death of a minor unless the**
20 **health care provider has received the consent of each parent or**
21 **each legal guardian of the minor. Consent by each parent or each**
22 **legal guardian must be given orally and in writing to the health**
23 **care provider. The health care provider must have at least two (2)**
24 **witnesses attest that the consent was given by each parent or each**
25 **legal guardian. The health care provider shall document the**
26 **consent in the minor's medical record, specifying each parent or**
27 **legal guardian who gave consent. A parent or legal guardian may**
28 **revoke the consent at any time either orally or in writing.**

29 (g) This subsection does not apply to a minor described in
30 section 3(a)(2) of this chapter. A health care provider may not
31 interfere with the efforts of a parent or legal guardian to obtain
32 other medical opinions or transfer the minor's care to another
33 health care provider. A health care provider may not hinder or
34 delay necessary medical measures for a minor, including a
35 tracheostomy or gastrostomy tube required to transfer a minor
36 patient's care to another health care provider. If the parent or legal
37 guardian of a minor requests a transfer for the minor patient's
38 care, the health care provider must continue to provide life
39 sustaining procedures, including nutrition and hydration, until the
40 transfer is made.

41 (h) This subsection does not apply to a minor described in
42 section 3(a)(2) of this chapter. A court does not have jurisdiction to



- 1 **withdraw life sustaining procedures from a minor over the**
- 2 **objection of a parent or legal guardian.**

